



Name : Mr. Vipin Kumar Garg
Age / Gender : 67 yr(s) / Male
Bed / Ward No. : 303 / 3F Special (Isolation)
Admitting / Consulting Doctor 1 : Dr.M Mashru and J Dobariya / Dr.M Mashru and J Dobariya

MR No : 152114
IP No. : 25167

Summary No. : DS-2024-0004345
Admission Dt / Tm : 07/09/2024 / 20:38:20
Discharge Dt / Tm : 09/09/2024 / 16:12:42

Diagnosis

LRTI with Respiratory Failure

Urinary Tract Infection

Pancytopenia

AKI

Hyponatremia

K/C/O:- Hypertension , Multiple Myeloma , Asthma , Hypothyroidism , Diagnosed as prediabetic , BPH

ICD Code	Name
J96.9	Respiratory failure, unspecified
N39.0	Urinary tract infection, site notspecified
I15	Secondary hypertension

Presenting Complaints

67 year old male patient brought to casualty from hotel by Ambulance with c/o: Fever since 4 days, Cough, Perspiration, Decrease urine output since 1 day, 1 episode of fainting yesterday and Similar episode today morning
k/c/o Multiple Myeloma , Recent H/O -Taken chemotherapy on Wednesday (04/09/2024) -6th cycle, With the above-mentioned chief complaints and general physical examination patient, was admitted in ward.

Systemic Examination

BP during Admission 170/96 mmHg during discharge 150/70 MM HG,CNS during Admission conscious and oriented during discharge CIONSIOUS,ORIENTED,CVS during Admission s1s2+ during discharge S1S2+,Pulse Rate during Admission 80/min during discharge 86/MIN,Respiratory Rate during Admission 22/min during discharge 30/MIN,SpO2 during Admission 98% on RA during discharge 95% ON 4L/MIN WITH NC,Temperature during Admission 97.8°F during discharge 98.4 F

All the Relevant Investigations are attached with the discharge summary

Hospital Course

All required blood and radiology investigations were done which were s/o:- LRTI with Respiratory Failure , Urinary Tract Infection , Pancytopenia AKI, Hyponatremia , and the treatment was started accordingly.
The patient's GC and prognosis were explained to the relatives in detail.

As Patient had H/O recent chemotherapy suspected for Febrile Neutropenia ,Higher antibiotic in combination started. Before antibiotic therapy Blood c/s ,Sputum c/s was final reports are awaited.

On 08/09/24 Urine c/s was sent which was s/o:Final No Growth

On 08/09/24 Sputum Biofire was sent which was s/o:Streptococcus Pneumoniae and treatment modify accordingly

On 07/09/24 In view of thrombocytopenia ,Dengue PCR was sent which was Negative.

On 07/09/24 CT Thorax done which suggest: Minimal bilateral pleural effusion is seen with fissural extension, passive atelectasis and mild smooth interlobular septal thickening of the underlying lung parenchyma, Few tiny focal areas of airspace consolidations are seen in basal segments of bilateral lower lobes (right > left).

On 07/09/24 in sputum Biofire Panel was sent which s/o: streptococcus pneumonia.

On 08/09/24 Cardiologist Dr.Shrenik Doshi reference done and his all advised followed.

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GSTIN : 24ACTFS3332C1Z8

LLPIN : AAD - 2783



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2D Echo done by Dr Shrenik Doshi ,which suggest – Normal LV function ,No RWMA ,LVEF-65% ,Type I OD ,RA/RV –Normal ,Tr –TR , ERVSP –30 , Sclerodegenerative AOV ,NO AS/AV ,Tr. MR ,Normal other valves, No clot/veg/PE, IVC—9-10 mm,collapsing.

On 08/09/24 In view of thrombocytopenia(PC - 18000) 1 Unit SDP transfused.

In view of AKI , Nephrologist (Dr.Niyati Bhalani) reference was done and her all advised were followed.

In view of tachypnea and hypoxia O2 support started with Nasal prongs and maintained saturation.

On 08/09/24 His foley's catheter insertion was done with all aseptic precautions.

On 08/09/24 Dr.Shweta Tilara (oncophysician) reference was done and her all advised followed.

On 08/09/24 his USG Abdomen and pelvis was done which s/o:Gall Bladder is partially distended and shows minimal sludge within lumen. No abnormal wall thickening noted.

Both kidney shows mild diffuse increased renal parenchymal echotexture suggestive of likely acute renal parenchymal disease,Prostate appears mild enlarged.Mild free fluid noted within interbowel region.

On 08/09/24 In view of thrombocytopenia(PC - 8000) 1 Unit SDP transfused.

On 09/09/24 In v/o Decrease Urine output Inj Lasix Infusion was started

On 09/09/24 In view of thrombocytopenia(PC - 18000) 4 Unit PC transfused.

On 09/09/24 Dialysis catheter insertion was done by Dr.Milap Mashru with all aseptic precaution without any intra and post procedure complication

On 09/09/24 x ray chest was done which was s/o: Rotation,HD catheter seen in situ,ill-defined opacity seen in right mid-lower and left lower zone with obliteration of both CP angles.

On 09/09/24 In v/o Decrease Urine output SLED Dialysis was done and advised by Dr.Niyati Bhalani(Nephrologist)

On 09/09/24 Patient Transfer to Higher Center for Further Management

On discharge(09/09/2024):-

T-97.7°F

P-84/MIN

BP-154/88 mmHg

RR- 26/MIN

Spo2 -94% with 4L oxygen support with NP

ECG:BLAE+

S:S1S2+

CNS:Conscious ,oriented

P/A:Soft,No Tenderness

Peripheral line – Day 1

EJV-Day-1

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Dialysis catheter-Day-1

Foley /urometer – Day 2

Treatment During Hospitalization

Drug Name

Cap Doxy-1 L-Dr Forte

Inj Sodium Bicarbonate 25 ML

Tab Telekast A

Inj Lasix 4 ML

Inj Emeset 2 ML

Tab Apresol

Inj Ticofic 400 MG

Respules Duo2 2.5 ML

Inj Gocan 100 ML

Inj Neukine 300 MCG

Inj Cortem 100 MG

Tab Rosuson 10 MG

Inj Bibonate 100 ML

Inj MVI

Inj Paracip 100 ML

Tab Zytanix 5 MG

Inj Paridem 40 MG

Tab Amlong 5 MG

Inj Mucad 1 GM

Advice on Discharge

Hospitalization-ICU Admission

Neutropenic care

Summary of Key Investigations

CRP (C Reactive Protein) (07/09/2024), S. Bilirubin (07/09/2024), S. Creatinine (07/09/2024), S. Potassium (07/09/2024), SGPT (07/09/2024), Urine Routine And Micro (07/09/2024), PCT (Procalcitonin) (07/09/2024), CBC (07/09/2024), MP By Card (07/09/2024), Blood Urea (07/09/2024), CT Chest Plain (08/09/2024), Portable Echocardiography (09/09/2024), BGRh (08/09/2024), S. Bicarbonate (08/09/2024), CBC (08/09/2024), NT Pro-BNP (08/09/2024), APTT (08/09/2024), PT (Prothrombin Time) (08/09/2024), MP By Card (08/09/2024), USG Abdomen With Pelvis (08/09/2024), S. Bicarbonate (09/09/2024), S. Calcium (09/09/2024), S. Creatinine (09/09/2024), S. Potassium (09/09/2024), S. Sodium (09/09/2024), S. Uric Acid (09/09/2024), CBC (09/09/2024), CRP (C Reactive Protein) (09/09/2024), PCT (Procalcitonin) (09/09/2024), HBsAg (09/09/2024), HCV (Hepatitis C) (09/09/2024), HIV I And II (09/09/2024), X Ray Chest - Portable (09/09/2024)



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Investigations Done

	07/09/2024	08/09/2024	09/09/2024
CRP	22.6		26.5
Malarial Parasite by card	Negative	Negative	
ABO Group		"B"	
Rh FACTOR		POSITIVE	
S. Bicarbonate		17.2	17.6
HBsAg (By Card)			Non
			Reactive
HBsAg (By ECLIA)			-NA-
HCV (By Card)			Non
			Reactive
HCV (By ECLIA)			-NA-

Mr. Vipin Kumar Garg

(RMO)

Dr. M Mashru and J Dobariya

In case of any emergency, kindly rush to our Emergency Department or contact at 0281 6195050